PROBLEM

What was happening — and why it mattered

WHAT WAS HAPPENING

The MPS invoice portal had PMF for small-volume providers (322K digital lines/year, 8.7% digital share) — but 40% of total invoice volume from high-throughput providers was architecturally blocked. Product limits: 1 claim per submission, 1 invoice reference, 11 screens, ~32 line items max.

WHO WAS AFFECTED

Worker travel (taxi) providers billing dozens of claims simultaneously; hearing aid providers with thousands of lines/month; high-volume physio and imaging clinics. Existing low-volume users were unaffected but growth had structurally stalled.

WHY IT MATTERED

25 high-volume providers (of 3,795 total) generated 50% of all remaining paper invoices. Without segment-specific product and GTM strategy, WorkSafeBC could not move mail/fax volume to digital — blocking operational savings and provider experience improvements.

DISCOVERY

How I approached discovery

ANALYTICS

3.7M total line items across all channels

8.7% digital share was healthy for the segment served — but the product had structurally run out of room to grow without architectural changes.

RESEARCH

Pareto analysis · provider segmentation

25 high-volume providers = 50% of all paper invoices. Targeted adoption on this cohort maximizes ROI vs. broad communications to 3,795 providers.

USER INTERVIEWS

High-volume provider workflow mapping

Providers weren't avoiding the product because it was hard to learn — they couldn't use it at all. A taxi company with 1,900 lines/month hit a wall immediately.

WORKFLOW OBSERVATIONS

V1 submission flow analysis

11 screens × 50 invoices = untenable workflow. Multi-claim and multi-invoice per session were structural requirements, not UX polish.

STAKEHOLDER INTERVIEWS

Healthcare Operations, program managers, Blueprint vendors

A single portal couldn't serve every segment — hearing aid providers need Blueprint bulk upload; physicians need EMR integration at point of care.

PRODUCT JUDGMENT

“We intentionally optimized for 25 high-volume providers instead of all 3,795 after Pareto analysis showed they generated 50% of remaining paper invoices.”

CURRENT WORKFLOW

Before — and where it broke

V1 workflow — structural ceiling blocked high-volume providers



PRODUCT JUDGMENT

“We diagnosed a capacity ceiling, not a UX problem — providers weren't avoiding the product because it was hard to learn; they couldn't use it at all.”

PRODUCT THINKING

Observation → insight → decision

DECISION CHAIN 1

OBSERVATION	INSIGHT	DECISION	TRADEOFF	EXPECTED OUTCOME
322K digital lines/year proved PMF for low-volume providers, but 40% of volume remained on paper.	This wasn't a UX problem — it was a capacity ceiling. The product was architected for physio clinics billing 3 claims/week.	Rebuild submission experience for high-volume segment: multi-claim, multi-invoice, 5 screens, no line ceiling.	Segment-specific build vs. incremental improvements for existing users who were already active.	Unlock 280K blocked paper line items; targeted GTM on 25 high-volume orgs representing 50% of paper volume.

DECISION CHAIN 2

OBSERVATION	INSIGHT	DECISION	TRADEOFF	EXPECTED OUTCOME
25 providers generated 50% of paper invoices — Pareto distribution.	Broad rollout risks disrupting existing users; named-cohort launch to highest-value converters maximizes ROI.	Dark launch to 10 named providers (Jan 1–14, 2026); V1 preserved throughout.	Slower broad activation vs. zero disruption to 322K-line existing user base.	Validate v2 with high-volume cohort before soft/hard launch; PostHog instrumentation from day one.

DECISION CHAIN 3

OBSERVATION	INSIGHT	DECISION	TRADEOFF	EXPECTED OUTCOME
Hearing aid providers run invoices through Blueprint — leaving EMR for portal solved little.	Different segments need different channels: portal redesign, vendor integration (Blueprint), or direct EMR (OSCAR).	Shift from single-product optimization to platform strategy with three distribution paths.	Broader roadmap scope vs. honest acknowledgment that one portal can't unlock all paper volume.	$580K funding for portal + interoperability roadmap + Blueprint bottom-up demand + OSCAR API validation.

DECISION CHAIN 4

OBSERVATION	INSIGHT	DECISION	TRADEOFF	EXPECTED OUTCOME
First month post-launch saw almost no adoption despite better features (11 → 5 screens, multi-claim).	Shipping better features isn't enough — discoverability, onboarding, and post-launch behavior observation matter.	Segment-specific GTM messaging; observe real user behavior; fix duplicate-line icon discoverability.	Additional change management investment vs. assuming feature quality drives adoption.	Learning that launch is the beginning, not the finish line — informed hard launch strategy.

DECISION CHAIN 5

OBSERVATION	INSIGHT	DECISION	TRADEOFF	EXPECTED OUTCOME
AI-assisted prototype validated 11→5 screen consolidation; legacy Kendo Grid couldn't support design cleanly.	Prototypes validate desirability, not feasibility — technical discovery must run alongside UX exploration.	Adapt design to Kendo Grid constraints while preserving core workflow reduction.	Compromised ideal UX vs. shippable solution within legacy stack limitations.	Delivered within Aug 2025 – Mar 2026 timeline despite legacy technology constraints.

PRODUCT JUDGMENT

"We chose a named-cohort dark launch over big-bang migration because the 322K-line existing user base must not be disrupted during validation."

KEY PRODUCT DECISIONS

Decision matrix

KEY PRODUCT DECISIONS

Decision	Alternative	Why we chose this	Tradeoffs	Outcome
Segment-specific rebuild over incremental UX polish CONFIGURATION VS PRODUCT	Improve existing portal for all users equally	40% of volume was architecturally blocked — incremental polish wouldn't unlock high-volume providers.	Two product experiences (V1 + V2) during transition period	V2 purpose for 200–1,9 lines/month preserved existing use
Targeted adoption on 25 high-volume providers (50% of paper) PROCESS VS SOFTWARE	Broad communications to all 3,795 providers	Pareto analysis showed 25 orgs represent half of remaining paper — focused GTM maximizes ROI.	Narrower initial activation vs. efficient use of change management resources	Dark launch worker trav hearing aid & imaging
Named-cohort dark launch; V1 live throughout SPEED VS SAFETY	Big-bang migration for all users	Existing 322K digital lines/year user base must not be disrupted during validation.	Dual maintenance of V1 and V2 during transition	Zero disrup existing use validated v hard launch 25, 2026
Platform strategy (portal + Blueprint + EMR) over single portal TECHNICAL DEBT VS SCALABILITY	Build all features into one portal	Hearing aid providers need Blueprint bulk upload; physicians need EMR integration — one product can't serve every workflow.	Broader roadmap and ecosystem engagement vs. simpler single-product narrative	$580K func interoperab roadmap; C API validate Blueprint b up demand
PostHog instrumentation spec before launch USER EXPERIENCE VS COMPLIANCE	Add analytics after launch based on assumptions	Need to know within days of soft launch whether new segment is activating and existing users disrupted.	Upfront instrumentation effort vs. flying blind post-launch	v1 vs v2 comparison day one: lir multi-claim large-volun activation

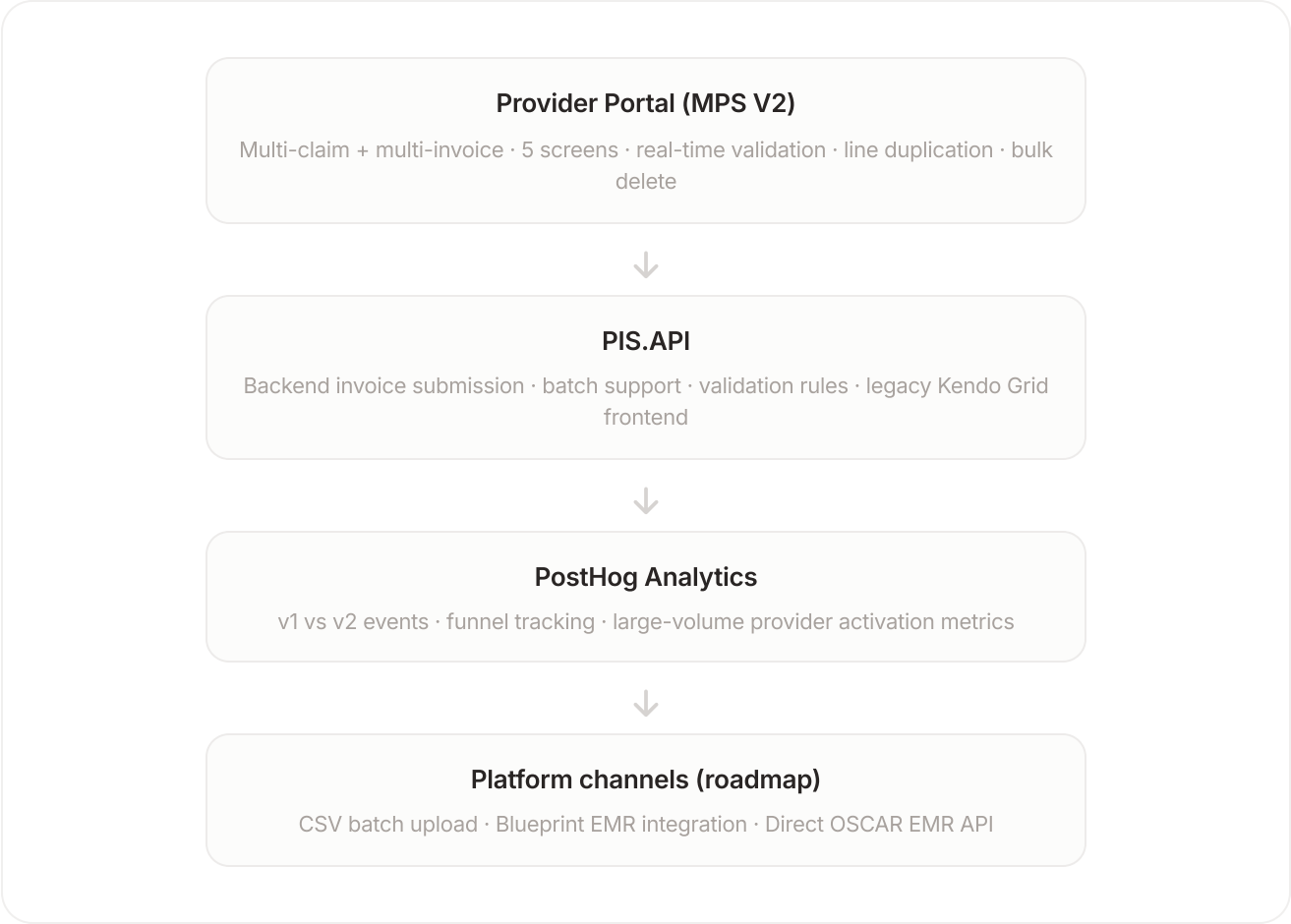
PRODUCT JUDGMENT

“We shifted to platform strategy (portal + Blueprint + EMR) because one product couldn't unlock all paper volume — honest segmentation beats universal feature claims.”

TECHNICAL COLLABORATION

Architecture at product level

Product-level architecture — segment expansion on legacy PIS platform



PRODUCT RESPONSIBILITIES

- Segment identification and Pareto-informed GTM strategy
- $580K business case and executive funding
- PostHog analytics spec before launch
- Platform strategy across three provider segments

ENGINEERING RESPONSIBILITIES

- PIS.API multi-claim and multi-invoice support
- V2 portal rebuild (11 → 5 screens)
- Real-time error validation and bulk delete
- Performance optimization for large sessions
- Legacy Kendo Grid adaptation

SHARED RESPONSIBILITIES

- Dark launch cohort selection with Healthcare Operations
- Blueprint bottom-up demand mobilization
- V1/V2 parallel operation during transition
- Retry queue implementation (learned from WOR project)

Named-cohort launch planning and change management

TECHNICAL TRADEOFFS

Legacy Kendo Grid constrained ideal single-screen design — adapted within feasibility

Capacity scaling without equal performance attention caused slowdowns at large session sizes

V1/V2 dual maintenance during transition — necessary to protect existing user base

Platform roadmap (Blueprint, EMR) deferred full paper elimination beyond portal segment

PRODUCT JUDGMENT

“We wrote the PostHog spec before launch because we needed to know within days whether the new segment was activating — not months after guessing.”

EXECUTION

How we shipped

TIMELINE

Aug 2025: Project kickoff · business cases · $580K funding secured

Aug 2025 – Mar 16, 2026: V2 development and delivery

Jan 1–14, 2026: Dark launch — 10 named high-volume providers

Jan 14+: Soft launch — V2 default, V1 via banner

Mar 12–25, 2026: Hard launch with batch upload live

PRIORITIZATION

Structural barriers first: multi-claim, multi-invoice, screen reduction

PostHog instrumentation before Slice 1 launch

Targeted GTM on 25 orgs (50% paper) over broad rollout

Platform channels (CSV, Blueprint, EMR) on roadmap after portal V2

MVP (V2 DELIVERED)

Multi-claim + multi-invoice per submission

11 screens → 5 screens

Real-time error validation

Line duplication and bulk delete

Targeted adoption strategy with PostHog measurement

RISKS

Near-zero adoption in first month despite better features

Performance degradation at large session sizes

Legacy Kendo Grid feasibility constraints

Segment-specific messaging neglected (hearing aid still leaving EMR)

TODO: hard launch batch upload risks

TRADEOFFS ACCEPTED

V1/V2 dual operation during transition

Segment-specific build over universal improvements

Design compromises for legacy stack

Phased GTM over big-bang migration

CROSS-FUNCTIONAL PARTNERS

Healthcare Operations (provider relationships)

Program managers (Blueprint bottom-up demand)

Executive sponsors ($580K approval)

Engineering (PIS.API, Carebears/Pioneers pods)

Direct Connect team (OSCAR EMR API validation)

PRODUCT JUDGMENT

“We invested in segment-specific change management after launch when adoption stayed near zero — shipping features wasn't enough without discoverability and onboarding.”

RESULTS

Outcomes

OPERATIONAL OUTCOMES

56% → 19%

PAPER CHANNELS (MAIL + FAX)

Combined paper share of all invoice line items — from majority to under a fifth (2023–2026)

USER OUTCOMES

39% → 55%

MPS PORTAL SHARE

Core online portal share grew as new high-volume segments activated on V2

BUSINESS OUTCOMES

$580K

FUNDING SECURED

Business cases approved by executive sponsor before development began

BUSINESS OUTCOMES

~$50K

ELAN SAVINGS TO DATE

Early savings from paper volume reduction as first cohorts moved to MPS v2

USER OUTCOMES

11 → 5

SCREENS PER INVOICE

Workflow reduction enabling viable high-volume submission sessions

OPERATIONAL OUTCOMES

44K

TARGET DIGITAL LINES/MONTH

Hard launch target — up from 26K/month baseline. TODO: post-launch actuals

Paper channels dropped from 56% to 19%; MPS portal grew from 39% to 55%

Avg line items per submission — v1 vs v2 (PostHog)

Did v2 activate larger-volume providers? (PostHog)

Monthly submission trend — v1 vs v2 (PostHog)

PRODUCT JUDGMENT

"We measured channel shift (paper 56%→19%) not just feature usage — the business goal was moving providers off mail and fax, not shipping V2."

LESSONS LEARNED

What changed how I think

What surprised me

First month after launch saw almost no adoption despite 11→5 screens and multi-claim. Users didn't recognize the duplicate-line icon and manually re-entered data — great features only create value if users discover them.

What I would do differently

Segment GTM messaging from day one — hearing aid providers needed Blueprint integration narrative, not portal features. We segmented the roadmap but not the launch communications.

How this changed how I think as a PM

One product can't serve every provider workflow. Pareto-informed targeting + platform thinking (portal, vendor, EMR) beats universal feature optimization — especially when 25 orgs represent 50% of the remaining problem.

WHY THIS MATTERS

Relevance to healthcare product leadership

Provider invoice submission sits at the intersection of B2B platform strategy, segment expansion, and healthcare operations. It required diagnosing a structural capacity ceiling (not a UX problem), securing executive funding with data-driven business cases, and building a measurement framework before launch — the same product leadership needed for healthcare workflow products where adoption, not functionality, is often the primary constraint.

QUESTIONS THIS PROJECT FORCED ME TO ANSWER

Head of Product questions — and how I answered them

Is this actually a software problem?

Yes — structural product limits (1 claim, 11 screens, 32-line cap) architecturally blocked an entire segment. But adoption also required GTM and change management, not just better software.

Which user segment actually mattered most?

25 high-volume providers generating 50% of paper invoices — worker travel, hearing aid, high-volume physio/imaging. Optimizing for the average provider wouldn't unlock growth.

What evidence changed our direction?

Pareto analysis (25 orgs = 50% paper); near-zero post-launch adoption despite better features; segment interviews showing Blueprint/hearing aid needed different channel than portal.

What tradeoffs did we accept?

V1/V2 dual operation; segment-specific build over universal polish; design compromises for legacy Kendo Grid; phased GTM over big-bang.

What would happen if we built nothing?

40% of invoice volume remains on paper; 280K line items blocked; operational costs from mail/fax processing continue; digital share stalls at 8.7%.

Should this be solved with process, configuration, software, or AI?

Software for portal rebuild (structural limits); platform strategy for segments beyond portal reach; process/GTM for adoption; AI assisted prototyping but legacy stack determined implementation.

IF I DID THIS PROJECT AGAIN

Knowing what I know today

What would I change?

Segment launch messaging alongside segment product development — different provider groups need different value propositions from day one.

What would I validate earlier?

Legacy Kendo Grid constraints alongside UX prototyping — technical feasibility discovery should run parallel to desirability validation.

What assumptions were incorrect?

Assumed shipping better features (11→5 screens, multi-claim) would drive adoption. Discoverability and onboarding were the actual constraints post-launch.

What would I intentionally NOT build?

Universal portal trying to serve hearing aid (Blueprint), high-volume (portal V2), and physicians (EMR) with one experience — platform honesty beats feature accumulation.

How would AI change my approach today?

AI-assisted prototyping compressed concept validation, but wouldn't replace PostHog instrumentation or segment-specific GTM — adoption constraints remain human.

How would I measure success differently?

Add performance metrics alongside capacity — large sessions became slower post-launch; scalability and responsiveness should be one problem.

What would I prototype before implementation?

Post-launch onboarding flow with real providers — paper prototype of duplicate-line feature discoverability before assuming icon-based UX would work.

HEAD OF PRODUCT VIEW

If you only read one thing — why trust this PM with a healthcare workflow?

PROBLEM

40% of invoice volume architecturally blocked — 25 high-volume providers generating 50% of paper invoices on a product built for 3 claims/week.

JUDGMENT

Diagnosed capacity ceiling, not UX problem. Pareto-informed targeting + platform strategy over universal portal optimization.

HARDEST TRADEOFF

V1/V2 dual operation during phased launch — protecting 322K-line existing user base while validating segment rebuild.

TECHNICAL CHALLENGE

Legacy Kendo Grid constraints on ideal UX; performance degradation at scale; multi-claim architecture on existing PIS.API.

HEALTHCARE COMPLEXITY

3,795 providers across worker travel, hearing aid, physio, imaging — each with different billing workflows and distribution channels.

BUSINESS OUTCOME

Paper 56%→19%; MPS portal 39%→55%; $580K funded; ~$50K ELAN savings; platform roadmap with OSCAR API and Blueprint demand.

PRODUCT LESSON

Launch is the beginning of learning — great features only create value if users can discover, understand, and confidently use them.